جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : زينب انور جبر عوض
المحافظة : الجيزه **الحي / المدينة :** اطفيح **العنوان :** منشاة سليمان
رقم البطاقة : 29510012105981 **رقم التليفون :** 01111955184 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 01-10-1995 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: زينب انور جبر عوض
National ID: 29510012105981

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Supportive and palliative Treatment**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient's Name:	**Zainab Anwar Gabr**
Date:	**2026-06-10**
Referred by:	Prof.Dr. no doctor
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA with no available previous DICOM images for comparison.

I- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple axial and appendicular skeleton predominantly lytic bone lesions.

Opinion:

- **Multiple axial and appendicular skeleton predominantly lytic bone lesions likely metastatic.**

Dr.sherif Wagdy

CT Scan
[[DOCTYPE:CT_SCAN]]

Name: زينب انور جبر	
MR. : 3146	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient has metastatic breast cancer to bone and has received a regimen consisting of TAXOL, HERCEPTIN, XELODA, and TYKERB. The patient experienced severe hand-foot syndrome attributed to XELODA, and treatment with TYKERB was discontinued after 18 cycles. Current supportive therapy includes FEMARA, ZOMETA, and ZOLADEX.

The patient continues to have metastatic breast cancer to bone and has been treated with TAXOL, HERCEPTIN, XELODA, and TYKERB. Severe hand-foot syndrome from XELODA led to discontinuation of TYKERB after 18 cycles. The patient remains on FEMARA, ZOMETA, and ZOLADEX while awaiting pending laboratory results.

According to the multidisciplinary team (MDT) discussion in October 2024, the patient was diagnosed in May 2022 with de novo metastatic breast cancer to bone, confirmed by calcified biopsy as metastatic adenocarcinoma. The patient received palliative radiation therapy followed by 16 weeks of TAXOL plus HERCEPTIN IV and monthly ZOMETA. A bone scan demonstrated progressive disease, prompting initiation of TYKERB plus XELODA in November 2022; however, severe hand-foot syndrome from XELODA resulted in a treatment-related adverse event (CTCAE). By June 2023, the disease was stable on ZOMETA, FEMARA, and ZOLADEX, and a bone scan in September 2024 remained stable. The patient completed all approved cycles of TYKERB (no further approval from the Ministry of Health). The MDT recommended continuation of FEMARA, ZOLADEX, and ZOMETA while awaiting pending bone scan and CT CAP results. The most recent clinic visit was on 12 July 2026.

The most recent assessment shows a bone scan performed on 18 June 2026 indicating stable disease, and a CT CAP on 10 June 2026 also stable. The patient is currently free to continue FEMARA, ZOLADEX, and ZOMETA, with additional supportive treatment including a proton pump inhibitor, calcium, bone-specific therapy, and methylprednisolone. Laboratory results are acceptable, and the pharmacy has been notified to dispense the prescribed medications. The current plan is to maintain FEMARA, ZOLADEX, and ZOMETA with the supportive regimen.

تحريرا في ,

التاريخ :	21-09-2026	**الوقت :**	
الطبيب المعالج :	هنا اسامه عبد الرحيم عبد البر		

يعتمد ,

Name : Zainab Anwar Gabr Awad
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 18-06-2026 10:39
Medical No. : 397003

Tc-99m MDP WHOLE-BODY SKELETAL SCINTIGRAPHY

- **Clinical History:** A 31-year-old female patient with a known history of right breast cancer. She is currently undergoing hormonal therapy and complains of bone aches. The scan is performed for follow-up and evaluation of therapeutic response, compared with the previous study dated 24-12-2025.
- **Technique:** Anterior and posterior whole-body planar scans were acquired 2 hours following the intravenous injection of Tc-99m MDP.

Findings

In comparison with the previous bone scintigraphy dated 24-12-2025:

- **Skeletal Metastases:** The previously described widespread active osseous lesions involving the sternum, bilateral ribs, multi-level spine, pelvic bones, scapulae, and proximal humeri demonstrate a **distinct reduction in both radiotracer intensity and spatial extent**, indicating a **regressive course.**
- No new focus points of pathologically increased radiotracer accumulation are detected throughout the rest of the skeleton.
- Stable degenerative features are noted in the spine and large joints.
- Normal physiological radiotracer distribution is seen in the kidneys and urinary bladder.

Impression

- **Regressive skeletal course** of the known widespread metastatic osseous lesions, indicating a favorable response to current therapy.
- No scintigraphic evidence of new skeletal metastases or active disease progression.
- Clinical and biochemical correlation is recommended as well as follow-up bone scan to monitor response.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : زينب انور خير عوض
Sex: Female
3- Medical number: 83078717
Old Med : 2047-2022
2-ID number:
Patient Age : 28 Y

MDT

Family history		Phone	01154077666 01111955184	BMI		REASON OF MDT	
marital status		NRS Score		GROUP		Date of MDT	08/10/2024
Co morbidity		Menopausal status		Initial STAGE			

Clinical data

Breast Imaging :

Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	8/8	8/8	3		

T	N	M
		1 bone

Sonomammography

PATIENT HISTORY

5/2022-->DENOVO MET RT BREAST TO BONE
CALCIFIED BIOPSY-->MET ADENOCARCINOMA,,RECEIVED PALLIATIVE RTH-->16 WK TAXOL
+HERCEPTN IV+ZOMETA MONTHLY+ZOLADEX
BONE SCAN-->PD STARTED TYKERB+XELODA FROM 11/2022 BUT
PT HAD SEVERE HAND-FOOT SYNDROME FROM XELODA
CTCAP-->OVERALL SD
6/2023:tykerb/zometa/femara/zoladex
9/2024 new bone scan is stable
pt completed 18 cycles of tyekrb (no more approval by MOH)

MDT OUTCOME & PLAN

continue femara ,zoladex ,zometa

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	8/10/24
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

HEALTH INSURANCE ORGANIZATION
CENTER DEPARTMENT OF
INFORMATION AND DECIION SUPPORT

الهيئة العامة للتامين الصحى
الادارة المركزية لتكنولوجيا المعلومات
ودعم القرار

تاريخ الاصدار :2022-12-10

جهه الاصدار :فرع الجيزة

الى من يهمه الامر .....

الهيئه العامة للتامين الصحى تعمل من خلال قوانين منظمة للانتفاع الخاضعين لقانون التامين الاجتماعى رقم 79 لسنه 1975 المعدل بالقانون 25 لسنه 1977 "القوى العامله والمعاشات والارامل " و كذلك قانون 99 لسنه 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنه 2012 " الاطفال دون السن المدرسى" وقانون رقم 23 لسنه 2012 الخاص بالمرأة المعيله .

وطيقا لقاعدة البيانات المرسلة من التأمينات الاجتماعيه

وطبقا لقاعدة البيانات المرسلة من الضمان الاجتماعى

والبحث تبين أن

السيد (ة) زينب انور جبر بطاقة رقم 29510012105981

وطبقا الى خطاب من التأمينات الإجتماعية يفيد عدم الأنتفاع
يعتبر من الفئات غير المنتفعه بالتامين الصحى
وشكرا

تاريخ الطباعه :2022-12-10

الموظف المختص

امتثال نعيم

افادة مركز نظم المعلومات
يعتمد،،،

مسلسل 244017

يعتبر هذا الخطاب سارى لمدة شهر من تاريخ الاصدار

الادارة المركزية لتكنولوجيا المعلومات و دعم القرار

Non-HIO Letter
[[DOCTYPE:NON_HIO_LETTER]]

Name : Zainab Anwar Gabr Awad
Branch : Maadi
Ref.Physician Prof. : No doctor
Date : 07-07-2025 02:09
Medical No. : 143968

Follow-up Whole Body (18F-FDG) PET/CT Study

History and Clinical Data:

Patient data:

- Weight: 78 Kg, Blood Glucose level: 75 mg/dl.

Clinical background:

- 29-year-old patient with history of managed metastatic right breast cancer.

Radiopharmaceutical:

- 159.1 MBq of 18F-FDG was injected I.V.

Purpose of the study:

- Follow-up considering the prior (FDG) PET/CT study report dated 31/12/2024.

Technique:

- Imaging was performed on an integrated PET/CT scanner approximately 60 minutes after injection of the radiopharmaceutical.
- Non-Contrast Enhanced CT images done for attenuation correction and anatomic localization were obtained followed by PET images from the vertex of the skull to the thighs.
- PET/CT images were reviewed in the transaxial, coronal and sagittal planes.
- The Standardized Uptake Value, maximum variant (SUV max) was calculated within regions of interest whenever needed.
- Reference **Hepatic SUV max ~ 3.9 and SUV mean ~ 2.2.**

Findings:

I) Locoregional (Both breasts and axillae):

- Tracer-starved right breast outer portion ill-defined soft tissue density, with no apparent FDG-avid parenchymal focal lesions detected.
- The left breast is unremarkable.
- No currently significant FDG-avid or pathologically enlarged axillary or other mammary-related lymph nodes.

II) Distant:

- **Rather stationary course of the priorly seen multiple low grade FDG-avid lytic osseous lesions scattered over the axial and appendicular**

PET Scan
[[DOCTYPE:PET_SCAN]]

Name : Zainab Anwar Gabr Awad
Branch : Maadi
Ref.Physician Prof : No doctor

Date : 07-07-2025 02:09
Medical No. : 143968

skeleton, eliciting SUV max up to ~ 2.8 at the left iliac bone. Low grade FDG-avid sclerotic osseous lesion is seen at the left calcaneus, eliciting SUV max ~ 2.9 (was up to ~ 3).

- No other distant FDG-avid focal lesions or pathological lymph nodes could be seen allover the surveyed body.

III) Rest of the body:

- No apparent brain focal lesions or pathologic tracer distribution.
- Few variable-sized low grade FDG-avid bilateral sub-mandibular and bilateral upper deep cervical lymph nodes, reaching ~ 1.2 cm and eliciting SUV max up to 3.7 cm at the left upper deep cervical group.
- Few tracer-starved bilateral lung scattered tiny nodules; likely non-specific, yet no FDG-avid pulmonary nodules, masses or infiltrates.
- No pleural or pericardial effusion, thickening or masses.
- Mildly enlarged liver with no FDG-avid focal lesions. No intra or extra-hepatic biliary radicle dilatation.
- Normal appearance of the spleen, pancreas, kidneys, suprarenals and pelvic structures.
- No pelvi-abdominal ascites or fluid collections.

IV) Side findings:

- IUD is seen in place.
- Spondylodegenerative changes are noted over the spine.
- Multiple atherosclerotic calcifications are noted within the large vessels.

NILE DIAGNOSTIC CENTER

PET Scan
[[DOCTYPE:PET_SCAN]]

Name : Zainab Anwar Gabr Awad
Branch : Maadi
Ref.Physician Prof : No doctor
Date : 07-07-2025 02:09
Medical No. : 143968

V) Normal physiological tracer bio-distribution is seen over the brain, glandular structures, heart, bowel and urinary tract.

Conclusion:

Follow-up whole body (FDG) PET/CT study for a managed metastatic right breast cancer patient shows:

- Non-metabolically active right breast outer portion ill-defined soft tissue density; likely remittent neoplastic.
- Stationary course of the multiple low grade metabolically active axial and appendicular residual metastatic osseous deposits.
- No newly developed metabolically active elsewhere allover the surveyed body.
- Follow-up whole body (FDG) PET/CT will be advisable as clinically warranted.

As Regards,

Dr. Moumen Katamesh, M.Sc
Nuclear Medicine

Dr. Yasser Mohamed
Professor of Nuclear Medicine

NILE DIAGNOSTIC CENTER

PET Scan
[[DOCTYPE:PET_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Zainab Anwar Gabr Awad	Date : 31/07/2024
Exam : C.T CHEST	Age : 28 Year(s)
Medical ID: 2047/2022	Pat. ID: 12442

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST & PARA NASAL SINUSES:

1. **PNS:**
   - Polypoid mucosal thickening of right maxillary antrum/retention cyst.
   - Clear rest of the scanned paranasal sinuses. No air-fluid levels.
   - Clear right concha bullosa.
   - No CT evidence of hyper-densities within the mucosal thickening or bone destruction/erosions.
   - Patent osteomeatal units and sphenoethmoidal recesses.
   - Intact bony boundaries.
   - Bowing of the nasal septum to the right.
   - Prominent inferior nasal turbinates.
   - Bulky adenoid soft tissue.

   N.B: Destructive lytic lesion of the clivus is noted

OPINION:

- **Right maxillary sinusitis/retention cyst. No CT evidence of fungal sinusitis.**

2. **Chest:**
   - **Compared to previous study dated 31-12-2023.**
   - Still noted middle lobe micro nodules and atelectatic band.
   - No pulmonary areas of ground glass attenuation, consolidation or cavitary lesions.
   - No CT evidence of pathological mediastinal lymph node enlargement.
   - No pleural or pericardial collection, pleural thickening or masses.
   - No cardiac gross abnormalities.
   - Still noted axial and appendicular skeleton lytic lesions.

OPINION:

- **Rather stationary course since last study.**

Dr. Ali Elsisy, MSc

Signature Prof. Menna Tallah Hanafy
05/08/2024 04:47

CT Scan
[[DOCTYPE:CT_SCAN]]